

Youth Behavioral Health Data Strategy

Summary of Research Findings

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Why This Research Matters

Every day across Estes Park, Poudre, and Thompson School Districts, students walk into classrooms carrying more than their backpacks. Some are anxious. Some are deeply sad. Some are managing things at home that no young person should have to manage alone. And most of them, because they are not acting out or causing trouble, stay invisible to the adults around them.

We know this is happening because the data tells us so. The 2023 Healthy Kids Colorado Survey, using a regional sample of 5,034 Larimer County high school students, found that one in four students felt so persistently sad or hopeless that they stopped doing normal activities, and nearly one in eight seriously considered suicide. These numbers are improving compared to 2021, and that improvement reflects real work happening in our schools and community. But the progress is uneven and unevenly distributed. Hispanic and Latino students in our region are experiencing persistent sadness and hopelessness at a rate nearly six percentage points higher than their White peers. Transgender students are experiencing mental health crises at rates that far exceed the county average. And without a coordinated data system, we cannot see these patterns clearly, we cannot track whether the work is reaching the students who need it most, and we cannot coordinate a meaningful response across three districts that are currently operating largely in isolation from one another.

The Larimer County community has already made a substantial commitment to behavioral health. In 2018, 61 percent of voters passed a 20-year sales tax dedicated to behavioral health services, which now generates \$24.5 million annually. That investment has produced real infrastructure, including the Longview Campus acute care facility, a new adolescent crisis stabilization unit for youth ages 12 to 17, and 77 community organizations receiving grant support. The county's 2026 to 2030 Strategic Plan explicitly names shared metrics as a priority. What is still missing is the school-level data infrastructure to complement that community investment: a shared way of measuring how students are doing, a common tool for identifying those who need support, and a coordinated system for connecting what three separate school districts know about the youth they serve.

This document summarizes the research and local data that inform the current project. It is organized around five questions the project team needs to answer:

- What should a coordinated school behavioral health data system look like?
- Why are so many students not getting the support they need?
- What screening tools are the strongest candidates for Larimer County?
- How do we make sure the data system works for every student, not just some?
- What is already happening in our community that we can build on?

Each section draws on verified research sources and, wherever possible, grounds the findings in

the specific context of Larimer County’s three school districts. Together the five sections address the four project questions in the scope of work: what existing data sources can anchor a shared metric, how current survey instruments compare in their constructs and psychometric properties, which screening tools are most compatible with CO-MTSS, and where the gaps and opportunities are in the current data ecosystem. The goal is not to produce an exhaustive literature review but to answer these questions in a way that directly informs the practical decisions ahead.

What a Coordinated Data System Should Look Like

Colorado’s Multi-Tiered System of Supports framework sets clear expectations for how school districts should collect and use behavioral health data. According to the state’s official practice profiles (Colorado Department of Education, 2025), districts are expected to collect, analyze, and disaggregate data at least twice a year using valid and reliable tools, maintain a written process for selecting and reviewing screening instruments, and use multiple measures to build a picture of the whole child rather than relying on a single data point. The framework also requires that districts provide annual written guidance to schools on the assessment tools being used and the process for making decisions based on the data collected.

These are not abstract standards. They describe a functioning system with specific components: a data management infrastructure that can store and organize results, a team process for reviewing data and making decisions, a tiered continuum of supports that data feeds into, and a feedback loop that allows districts to evaluate whether their interventions are working. When all of these components are in place and working together, the MTSS framework enables schools to move from a reactive model, where support is provided only after a student reaches a crisis point, toward a proactive model, where support is offered early based on what the data shows.

None of our three districts currently meets these standards in a coordinated way. The 2024 Tri-District Report (Groves, Jorgensen, & Olsen, 2024) confirmed this directly. Each district uses different terminology, different tools, and different collection schedules, making cross-district comparison essentially impossible and regional planning very difficult. Counselors are carrying caseloads of 244 to 266 students per counselor, which means they have limited capacity to develop and maintain the kind of systematic data processes the framework envisions. Funding is inconsistent and, in some areas, unsustainable. And there is no shared mechanism for aggregating what the three districts collectively know about the behavioral health of Larimer County youth.

It is worth noting that this situation is not unique to Larimer County. None of the three districts are State Personnel Development Grant grantees, which means they are implementing COMTSS without the formal CDE technical assistance and coaching infrastructure that grantee districts receive. They are doing this work largely on their own, with limited external support, and with the significant resource constraints that characterize most rural and suburban school districts in Colorado. That context matters for how recommendations are framed and for what is realistic to ask of districts in the near term.

The COMTSS Regional Implementation Guide (Colorado Department of Education, 2024) makes clear that addressing cross-district coordination is exactly the role of a regional body like the MHSU Alliance. The regional role is to support and coordinate rather than mandate, helping districts align language, tools, and processes over time in ways that respect their autonomy and work within their capacity. Other Colorado regions have built shared data infrastructure using this approach, establishing common definitions, piloting shared instruments, and creating regional aggregation processes through voluntary collaboration rather than top-down requirements. The guide provides a practical roadmap for doing the same in Larimer County.

The starting point for any meaningful alignment is a shared language. The RtI Glossary of Terms (Colorado Department of Education, 2025) provides the official Colorado definitions for the terms that matter most to this work: screening, universal screening, validity, reliability, disaggregated data, evidence-based practices, and fidelity. These definitions exist because different organizations and different districts have historically used these words to mean different things, producing confusion and misalignment even when people believe they are talking about the same approach. Before the three Larimer County districts can align their data practices, they need to agree on what they mean by the key terms that will govern those practices. This glossary is the foundation for that conversation, and it will be the reference point for the survey crosswalk work in the next phase of the project.

The practical implication. The MHSU Alliance does not need to mandate anything. Its role is to bring the three districts to a common language and a common standard, using the CO-MTSS framework as the shared reference point. The current project is the first structured step in that process, and it is explicitly designed to proceed regardless of how quickly or slowly the districts are able to engage.

Why So Many Students Are Not Getting Support

The research on this question is both consistent and sobering. Weist and colleagues (2018) found that roughly one in five young people experience mental health challenges, but only 36 percent of those who need intervention actually receive it. That is a gap of almost two thirds. The question is why, and the answer has important implications for how we design the data systems and tools this project is meant to produce.

The core reason is structural. Most school data systems were built to catch students who act out and disrupt. Office referrals, discipline records, attendance flags, and teacher reports all tend to surface the students whose struggles are visible and external. But the students with depression, anxiety, and trauma do not usually come to the principal's attention. They sit quietly, stop participating, and slowly disappear from the life of the school. Weist et al. describe these as students with internalizing problems: students whose difficulties are internal rather than disruptive, and who are therefore systematically invisible to the tools that schools currently use to identify need.

Consider the difference between a student who gets into a fight in the cafeteria and a student who stops eating lunch because she cannot manage the social anxiety the cafeteria triggers. The first student will have a disciplinary record by the end of the day. The second student may go unnoticed for months or years. Both students need support. Only one of them is likely to get it under a referral-based system.

Weist et al. also point out that this problem is compounded by the way schools tend to allocate mental health resources. Because externalizing students generate referrals, they receive a disproportionate share of counselor time and intervention resources. Students with internalizing problems, who generate no referrals because their struggles are quiet, receive far less support even though their long-term outcomes, including risks of academic failure, social isolation, and suicide, are comparably serious. In some cases, research suggests, the long-term consequences of unaddressed internalizing problems are more severe than those associated with externalizing behaviors, precisely because they are not caught early.

The only reliable way to close this gap is to stop waiting for students to surface themselves through behavior and to start systematically looking for all students who are struggling. This is what universal screening at Tier 1 of the MTSS framework means in practice: every student, not just the ones who come to attention, is assessed on a regular schedule using a validated tool that can identify both risk and protective factors across the full population.

Barrett, Eber, and Weist (2013) describe the broader structural fix in their Interconnected Systems Framework. The ISF argues that Positive Behavioral Interventions and Supports (PBIS), a school-wide framework for preventing problem behavior and promoting positive school culture and school mental health should not operate as separate system, each with its own data, its own teams, and its own decision-making processes. They should be one coordinated system, using shared data to guide decisions at every tier. In the ISF model, school-wide behavioral data from the PBIS system is complemented by mental health screening data from the school mental health system, and both feed into a single team process that makes decisions about what supports students need and at what level of intensity. When this integration is working well, data flows across the system, students are identified earlier, and resources go where they are most needed rather than where the noise is loudest. This framework is available free through the PBIS Center at pbis.org and maps directly onto the coordination structure the MHSU Alliance is trying to build across our three districts.

It is also worth naming what the current situation costs. The 2024 Tri-District Report documented that 25.2 percent of Larimer County high school students felt persistently sad or hopeless in 2023, and 11.9 percent seriously considered suicide. These are students who are in our schools right now. Many of them are not receiving support because the systems their schools rely on were not designed to find them. Building a coordinated data system is not a bureaucratic exercise. It is the most direct way available to us to make those students visible and to connect them with the support they need.

The practical implication. A shared metric and a universal screening tool are not nice-to-haves. They are the specific mechanisms by which students who are currently invisible to the system become visible. Without them, the work of supporting youth behavioral health in Larimer County will continue to be reactive rather than preventive.

Which Screening Tools Are the Strongest Candidates

Choosing a universal screening tool is one of the most consequential decisions in this project. The wrong tool, one that is technically inadequate, culturally inappropriate, or too burdensome for already-stretched school staff to administer reliably, will produce data that misleads rather than guides. The right tool will give school teams a clear, actionable picture of where students are and what they need.

Glover and Albers (2007) provide the most widely cited framework for making this decision, organized around three criteria. The first is appropriateness: is the tool designed for the school setting rather than a clinical one, does it measure the constructs that are most relevant to the local context, and is it suitable for the specific age groups and populations being served? The second is technical adequacy: does the tool consistently produce reliable results, does it accurately identify students who need support without systematically over-identifying or under-identifying, and is the evidence base strong enough to trust the results? The third is usability: can school counselors and staff realistically administer the tool within the existing school day, is the cost manageable, and does the data it produces lead directly to decisions and actions rather than sitting unused? Glover and Albers also flag that any tool must include appropriate accommodations for students with disabilities and students who are English Learners, a requirement that is directly relevant given the demographics of all three Larimer County districts.

Two tools stand out as the strongest candidates when these criteria are applied to our specific context.

The BASC-2 BESS. The Behavior Assessment System for Children, Second Edition Behavioral and Emotional Screening System (Kamphaus & Reynolds, 2007) is a 30-item screening tool for students in grades 3 through 12 that takes approximately five minutes to complete. It measures four areas: inattention and hyperactivity, internalizing problems such as depression and anxiety, school problems including academic concerns and negative attitudes toward teachers, and personal adjustment factors such as social stress. Results place students into one of three risk categories, normal, elevated, or extremely elevated, that map directly onto the tiers of the MTSS framework. This direct alignment with tiered decision-making is one of the BESS's most practical advantages: the output is immediately usable by school teams that are already operating within the MTSS model.

The tool's psychometric properties are strong. Split-half reliability scores of .90 to .93 and test-retest reliability of .80 indicate that the BESS produces consistent, trustworthy results across time and across different scorers. It has been validated across diverse student populations in

multiple contexts, including both urban and rural settings.

The most directly relevant validation study for our purposes is Dowdy and colleagues (2015), who implemented the BESS in two large urban high schools where 87 to 93 percent of students were Latino, demographics that closely resemble the populations in our three districts. When all 2,240 students were screened rather than only those referred through the existing system, 13 to 14 percent were identified as being at elevated or extreme risk. These were students who, under a referral-only approach, would have gone undetected and unsupported. The process also produced a meaningful shift in school culture: staff began discussing prevention strategies and population-level trends rather than only responding to individual students in crisis.

The Strengths and Difficulties Questionnaire. The SDQ (Goodman, 1997) assesses 25 attributes across five scales: emotional symptoms, conduct problems, hyperactivity and inattention, peer relationship problems, and prosocial behavior. It is available in parent, teacher, and self-report versions. The self-report version is appropriate for students aged approximately 11 to 17, covering the middle and high school years that are the primary focus of this project.

The SDQ's most significant practical advantage is accessibility. The questionnaire itself is freely downloadable from sdqinfo.org in more than 80 languages, including Spanish and Spanish adapted for the Rio de la Plata region. Given that Spanish is the primary home language for a large proportion of families across all three of our districts, and given the English Learner rates of 3 to 16 percent across the three schools, the availability of a validated Spanish-language version is not a minor convenience. It is a meaningful equity consideration. Computer scoring through SDQscore.org requires a license, and districts should factor this cost into their planning, but even with the scoring cost, the SDQ represents a significantly lower total cost than most commercially developed screening instruments.

The evidence base for the SDQ is extensive: 9,772 published studies across 123 countries as of 2024 (sdqinfo.org), encompassing diverse cultural, linguistic, and socioeconomic populations. In community samples, the SDQ has been shown to predict the presence of a psychiatric disorder with good specificity and moderate sensitivity, meaning it is reliable at correctly identifying students who do not have significant needs while also catching most of those who do.

Both tools meet the Glover and Albers criteria for appropriateness, technical adequacy, and usability when applied to the Larimer County context. The choice between them will depend on factors that the next phases of the project will examine in more detail: the specific demographics and capacity of each district, the cost implications for each option, the compatibility of each tool with existing data systems, and the preferences of the school staff who will administer it. What the research makes clear is that both tools are sound choices, and that either one would represent a significant improvement over the current situation, which is that no systematic behavioral health screening is happening at all.

Understanding why psychometric properties matter is also essential groundwork for the survey crosswalk that follows this bibliography. When the project compares survey instruments across the three districts, the central question is whether different surveys are actually measuring the same underlying constructs, even when their wording and format differ. DeVellis (2017) estab-

lishes that two instruments can appear to ask similar questions while measuring fundamentally different things, and that construct validity evidence is the primary way to determine whether items from different surveys can be meaningfully compared. Furr (2021) adds that reliability, particularly test-retest reliability across administrations, determines whether an instrument produces stable results that can be trusted as a baseline for tracking change over time. Dillman, Smyth, and Christian (2014) show that even small differences in question wording or response scale format can produce systematically different results, which means the crosswalk must examine not just what each district survey asks but exactly how it asks it. Finally, Boateng and colleagues (2018) highlight that cross-cultural validity, whether an instrument measures the same construct equivalently across different linguistic and cultural groups, is essential for any tool used in a community where Spanish is the primary home language for a significant share of families. These four methodological considerations will frame the construct-level and item-level analysis in the crosswalk phase of the project.

The practical implication. The BESS offers stronger integration with MTSS decision-making frameworks and has been directly validated with Latino-majority student populations similar to ours. The SDQ offers greater linguistic accessibility and lower upfront cost, which matters for resource-constrained districts like Estes Park. The final recommendation will weigh these tradeoffs against what is realistic for each specific district.

Making Sure the Data Works for Every Student

The research on equity in school data systems delivers a clear and uncomfortable message: when schools collect data but report only averages, they consistently produce numbers that look acceptable while concealing significant harm to specific groups of students. This is not a theoretical concern. It is documented repeatedly in the research literature, and it shows up in our own local data.

The 2023 HKCS Region 2 Report reveals disparities within Larimer County that a county-wide average would completely obscure. Hispanic and Latino students reported persistent sadness and hopelessness at 29.6 percent, compared to 23.6 percent for White students, a gap of six percentage points. Hispanic and Latino students were also less likely to report having a trusted adult at school: 75.2 percent compared to 81.9 percent for White students. Transgender students reported persistent sadness and hopelessness at 66.4 percent and seriously considered suicide at 50.0 percent, rates that are catastrophically higher than the county average. These students are in our three districts right now. A system that reports only county-wide averages will never see them.

Bal, Betters-Bubon, and Fish (2019) provide the most rigorous study of what happens when behavioral health data is not disaggregated. Their analysis of data from over 429,000 students in Wisconsin found that African American students were seven times more likely, and Latino and Native American students were two times more likely, to receive exclusionary discipline compared

to White peers. These disparities held even after accounting for student behavior, income, and English Learner status, meaning language proficiency was not the explanation for the gap. The only way these patterns became visible was through disaggregation. Schools that reported only school-wide averages never knew the disparities existed within their own data.

Importantly, Bal and colleagues also found that school-level factors mattered more than individual student characteristics in predicting behavioral outcomes. Schools with higher percentages of transferred students showed much higher rates of disproportionate outcomes, and this school-level pattern was more predictive than any individual demographic variable. This finding connects directly to Skiba and colleagues (2014), who examined over 104,000 disciplinary incidents across 730 schools and found that the strongest predictor of racial disparities in school outcomes was not student behavior or demographics but school-level variables, specifically the attitudes of principals toward discipline and the culture of the school.

Together, these findings have two specific implications for how this project designs the shared metric and the screening tool recommendation. First, any data system that only reports at the individual student level will miss the systemic patterns that drive inequitable outcomes. The shared metric needs to produce data that can be examined at the school level and the district level, not just the student level, so that the MHSU Alliance and district leaders can identify where systems are working well and where they are not. Second, the data must be disaggregated by race, language, and income as a standard practice, not as an optional analysis. Without disaggregation, the data will always tell a story about the average student rather than about the students who are most at risk.

There is also a tool-level consideration. Not all screening instruments perform equally well across different cultural and linguistic groups. A tool that was validated exclusively on White, English-speaking, middle-class student populations may produce systematically biased results when applied to diverse populations, over-identifying certain groups as high risk or under-identifying others. This is one reason why the SDQ's extensive international validation base across 123 countries is relevant: it provides evidence that the tool functions appropriately across culturally and linguistically diverse populations. The BESS's validation study in predominantly Latino schools (Dowdy et al., 2015) serves a similar purpose for that tool. When making the final screening tool recommendation, the cultural and linguistic validity of the instrument for Larimer County's specific student populations will be an explicit criterion.

Finally, it is worth naming the connection between equity and the ethical requirements of this project. The statement of work specifies that equity must be centered in all recommendations and that the project must avoid reinforcing bias in screening practices. These are not aspirational commitments. They are concrete design requirements that follow directly from what the research shows about what happens when data systems are built without equity as a foundational consideration.

The practical implication. Equity is not a separate consideration that gets added at the end. It has to be designed into the shared metric and the screening tool recommendation from the beginning, which means building in disaggregation by race, language, and income as a default, selecting tools that have been validated for diverse populations, and examining data at both the individual student level and the school and system level.

What Is Already Happening in Our Community

The research context matters, but so does the local one. Understanding what is already in place in Larimer County is essential for making recommendations that are grounded in reality and that build on existing strengths rather than proposing systems that would need to be built from scratch.

The Healthy Kids Colorado Survey. The most important existing data source for this project is the Healthy Kids Colorado Survey. Administered biennially by the Colorado School of Public Health, the HKCS is the state's most comprehensive survey on youth health and well-being. In 2023, 5,034 Larimer County high school students participated, and the results are available in a dedicated regional report (HKCS High School Region 2) that breaks down findings by gender, grade, race and ethnicity, sexual orientation, and gender identity.

The 2023 Larimer County results, verified directly from the official HSR 2 regional PDF, give us the clearest picture currently available of youth behavioral health in our community.

Larimer County 2023 (verified from the official HKCS High School Region 2 Report, 5,034 students)

- 25.2% felt so sad or hopeless almost every day for two weeks that they stopped usual activities (down from 41.3% in 2021)
- 11.9% seriously considered attempting suicide in the past year
- 9.9% made a specific plan about how they would attempt suicide
- 5.4% attempted suicide one or more times
- 15.1% purposefully hurt themselves without wanting to die
- 25.4% had poor mental health most or all of the past 30 days
- 79.9% have a trusted adult they can go to with a serious problem
- 63.3% said their stress level is manageable most days

Within-county equity disparities:

- Hispanic and Latino students: 29.6% felt persistently sad or hopeless vs. 23.6% for White students
- Hispanic and Latino students: 75.2% have a trusted adult vs. 81.9% for White students

- Transgender students: 66.4% felt persistently sad or hopeless; 50.0% seriously considered suicide
- LGBTQ+ students showed significantly worse outcomes across every mental health indicator

Two things are notable about these numbers. First, the progress since 2021 is real and meaningful. The sadness and hopelessness rate dropped from 41.3 percent to 25.2 percent, and the percentage of students reporting manageable stress levels increased. Something is working in our community, and it is worth acknowledging. Second, the within-county disparities are significant enough that county-wide averages tell a fundamentally misleading story. The average looks better than the reality for students who are Latino, transgender, or LGBTQ+. Any shared metric built on this data must be designed to surface these disparities, not average them away.

The HKCS is already administered across all three of our districts, it is free, and it has produced comparable data over multiple survey cycles. This makes the sadness and hopelessness item, which has been consistently tracked since at least 2019, the strongest existing candidate for a shared cross-district metric. The trusted adult item is also a strong candidate because of its direct relevance to school-based protective factors and its consistent improvement over time, which suggests it may be sensitive to school-level interventions that are already working.

The county's behavioral health investment. Larimer County has made a long-term, voter-backed commitment to behavioral health infrastructure. In 2018, 61 percent of voters passed a 20-year behavioral health sales tax that now generates \$24.5 million annually (Larimer County Behavioral Health Services, 2025). The 2025 Annual Report documents the concrete results of that investment: the Longview Campus acute care facility opened in December 2023 and now serves approximately 622 visits per month, the new adolescent crisis stabilization unit for youth ages 12 to 17 opened in 2025, and \$17.7 million in grants has been awarded to 77 community organizations since the program began.

The county also maintains a public behavioral health data dashboard at larimer.gov/health that includes Youth Emergency Mental Health Visit tracking and a Suicide Dashboard. These dashboards provide community-level data on the downstream consequences of untreated behavioral health needs, including emergency department visits and suicide-related contacts. They represent an additional layer of local data that this project can connect to, helping to build a more complete picture of how school-level behavioral health outcomes connect to community-level health system utilization.

The BHS 2026 to 2030 Strategic Plan explicitly lists shared metrics as a county-level priority. This means the data infrastructure work this project is doing is not happening in isolation from the broader county behavioral health system. It is directly aligned with what the county is trying to build, and the shared metrics this project recommends could inform not only what school districts track internally but what the county tracks at the community level.

Looking ahead: analytical methods that can strengthen this work. As the cross-district data infrastructure grows more complete and consistent, there is real potential to apply

more sophisticated analytical methods to strengthen both the shared metric and the screening tool recommendations. Shatte, Hutchinson, and Teague (2019) reviewed 300 studies applying machine learning to mental health data and identified two areas most relevant to this project. First, supervised classification methods can identify students at elevated risk more accurately than traditional cut-off scores, which has direct implications for how screening tool results are interpreted and acted on. Second, clustering methods can reveal whether items from different survey instruments are actually measuring the same underlying construct, which strengthens the crosswalk analysis. The review also flags an important equity consideration: models trained on non-representative data can amplify existing disparities rather than surface them, which reinforces the need to build equity in from the beginning rather than treating it as a filter applied after the fact. These methods align with the statistical and methodological background the intern brings to this project and represent a natural analytic extension as the data infrastructure matures.

The practical implication. The foundation is here. The funding is here. The community commitment is here. The HKCS gives us a starting point for a shared metric that is already in use and already free. The county's behavioral health infrastructure gives us a partner ecosystem that this project can connect to and build on. What is missing is the coordinated school-level data infrastructure to make all of it work together. Closing that gap is what this project is designed to do.

Where This Takes Us Next

The research reviewed in this document points toward four practical conclusions that will inform the rest of the project.

First, the Healthy Kids Colorado Survey is the strongest existing foundation for a shared cross-district metric. It is already administered across all three districts, it is free, and it has produced comparable trend data across multiple survey cycles. The sadness and hopelessness item and the trusted adult item are the strongest specific candidates for a shared indicator, given their consistency over time and their direct relevance to behavioral health outcomes. Before finalizing a recommendation, the next phase of the project will examine whether these items produce meaningful district-level variation and whether they are sensitive enough to reflect changes resulting from district-level interventions.

Second, a universal behavioral health screening tool is a genuine gap in the current system. Neither the HKCS nor the student perception surveys currently used by the districts functions as a Tier 1 behavioral health screener. The BASC-2 BESS and the Strengths and Difficulties Questionnaire are the two strongest candidates for addressing this gap, and the final recommendation will weigh their relative fit against what is practical and realistic for each specific district. The evaluation will use the three-part framework from Glover and Albers (2007): appropriateness for the local population, technical adequacy, and usability within existing school

structures.

Third, equity must be designed into both recommendations from the beginning. The within-county disparities documented in the 2023 HKCS data, and the broader research literature on how racial and linguistic disparities are obscured by aggregate reporting, make clear that a system designed around county-wide averages will fail the students who need it most. Disaggregation by race, language, and income must be a standard feature of whatever shared metric is recommended, not an optional analytical add-on.

Fourth, the MHSU Alliance’s role is to coordinate and support, not to require. The research confirms that this is both the appropriate role for a regional body under the CO-MTSS framework and the approach most likely to build the trust and voluntary buy-in that sustained cross-district alignment requires. The recommendations produced by this project should be framed as resources and tools that districts can adopt, with clear evidence for why they are worth adopting, rather than requirements imposed from above.

These four conclusions will carry forward into the survey crosswalk, the metric alignment analysis, the screening tool evaluation, and the final synthesis. The goal is not to produce a perfect system on paper but to give the MHSU Alliance, the Youth Mental Health Task Force, and the three school districts the practical foundation they need to take meaningful, coordinated steps toward a system that actually finds the students who are struggling and connects them with support.

References

- Bal, A., Betters-Bubon, J., & Fish, R. E. (2019). A multilevel analysis of statewide disproportionality in exclusionary discipline and the identification of emotional disturbance. *Education and Urban Society*, 51(2), 247–268. <https://doi.org/10.1177/0013124517716260>
- Barrett, S., Eber, L., & Weist, M. D. (2013). *Advancing education effectiveness: An interconnected systems framework for positive behavioral interventions and supports and school mental health*. Center for Positive Behavioral Interventions and Supports, U.S. Department of Education. University of Oregon Press. <https://www.pbis.org/resource/advancing-education-effectiveness-interconnecting-school-mental-health-and-school-wide-positive-behavior-support>
- Boateng, G. O., Neilands, T. B., Frongillo, E. A., Melgar-Quinonez, H. R., & Young, S. L. (2018). Best practices for developing and validating scales for health, social, and behavioral research: A primer. *Frontiers in Public Health*, 6, 149. <https://doi.org/10.3389/fpubh.2018.00149>
- Colorado Department of Education, Office of Learning Supports. (2024). *COMTSS regional implementation guide*. <https://www.cde.state.co.us/COMTSS/regional-implementation-guide>
- Colorado Department of Education, Office of Learning Supports. (2025, February). *Response to Intervention glossary of terms* (Version 2).
- Colorado Department of Education, Office of Learning Supports. (2025, December). *Colorado Multi-Tiered System of Supports district and school practice profiles* (Version 1). <https://www.cde.state.co.us/mtss>
- Colorado School of Public Health. (2023). *2023 Healthy Kids Colorado Survey high school Region 2 report: Larimer County*. Colorado Department of Public Health and Environment. <https://spl.cde.state.co.us/artemis/uchsmonos/uchs452h342023internet/>

- Dowdy, E., Furlong, M., Raines, T. C., Boverly, B., Kauffman, B., Kamphaus, R. W., Dever, B. V., Price, M., & Murdock, J. (2015). Enhancing school-based mental health services with a preventive and promotive approach to universal screening for complete mental health. *Journal of Educational and Psychological Consultation*, 25(2-3), 178–197. <https://doi.org/10.1080/10474412.2014.929951>
- DeVellis, R. F. (2017). *Scale development: Theory and applications* (4th ed.). SAGE Publications.
- Dillman, D. A., Smyth, J. D., & Christian, L. M. (2014). *Internet, phone, mail, and mixed-mode surveys: The tailored design method* (4th ed.). Wiley.
- Furr, R. M. (2021). *Psychometrics: An introduction* (3rd ed.). SAGE Publications.
- Glover, T. A., & Albers, C. A. (2007). Considerations for evaluating universal screening assessments. *Journal of School Psychology*, 45(2), 117–135. <https://doi.org/10.1016/j.jsp.2006.05.005>
- Goodman, R. (1997). The Strengths and Difficulties Questionnaire: A research note. *Journal of Child Psychology and Psychiatry*, 38(5), 581–586.
- Groves, H., Jorgensen, M. J., & Olsen, R. (2024). *2024 district school behavioral health assessment: Tri-district report*. Health District of Northern Larimer County; North Colorado Health Alliance; Alliance for Suicide Prevention.
- Kamphaus, R. W., & Reynolds, C. R. (2007). *Behavior Assessment System for Children, Second Edition: Behavioral and Emotional Screening System (BASC-2 BESS)*. Pearson.
- Larimer County Behavioral Health Services. (2025). *2025 annual report*. <https://www.larimer.gov/behavioralhealth>
- Larimer County Behavioral Health Services. (n.d.). *About behavioral health services and community master plan 2.0*. <https://www.larimer.gov/behavioralhealth/ourstory>
- Shatte, A. B. R., Hutchinson, D. M., & Teague, S. J. (2019). Machine learning in mental health: A scoping review of methods and applications. *Psychological Medicine*, 49(9), 1426–1448. <https://doi.org/10.1017/S0033291719000151>
- Skiba, R. J., Chung, C. G., Trachok, M., Baker, T. L., Sheya, A., & Hughes, R. L. (2014). Parsing disciplinary disproportionality: Contributions of infraction, student, and school characteristics to out-of-school suspension and expulsion. *American Educational Research Journal*, 51(4), 640–670. <https://doi.org/10.3102/0002831214541670>
- Weist, M. D., Eber, L., Horner, R., Splett, J., Putnam, R., Barrett, S., Perales, K., Fairchild, A. J., & Hoover, S. (2018). Improving multitiered systems of support for students with internalizing emotional/behavioral problems. *Journal of Positive Behavior Interventions*, 20(3), 172–184. <https://doi.org/10.1177/1098300717753832>